

Washington vs. Minnesota: Structural Comparison Matrix

Evidence Base Extraction: Section G

Status note: This is a supporting evidence-base memo from the project's early exploratory phase. Its framing predates the guardrail language later consolidated in `GUARDRAILS.md` and the manuscript, and an earlier draft of this table used stronger language than the evidence supports (see revision note below). It is retained for transparency and provenance. `GUARDRAILS.md` and the manuscript are canonical for framing; where this memo and those documents differ, the canonical documents control.

This matrix compares structural features of Washington's Adult Family Home (AFH) and HCBS programs against the oversight vulnerabilities documented in Minnesota's public Medicaid enforcement record. The comparison is structural, not evidentiary: it does not assert that any Washington facility or operator has engaged in the conduct described in Minnesota's enforcement cases. No fraud is alleged against any Washington provider anywhere in this document.

Structural Domain	Minnesota (HSS / HCBS / PCA)	Washington (AFH / HCBS)	Structural Comparison
Entry Barriers	Very low , per Minnesota audit findings. Housing Stabilization Services allowed billing shortly after basic registration, with minimal physical-site verification.	Comparatively high . AFHs require physical property, fire/health inspections, zoning approval, and DSHS licensing before Medicaid contracting.	Washington's licensing model does not share Minnesota's low-barrier registration structure. This is a genuine structural difference, not a review-priority signal.
Provider Growth	Rapid , per Minnesota audit findings. HSS billing grew from roughly \$2.6M to \$104M within three years; EIDBI from roughly \$600K to over \$400M.	Rising since 2018 . The licensed AFH count was roughly flat for a decade near 2,800, then grew to roughly 6,075 by 2026; the state LTC budget has grown alongside it (see <code>wa_data_aggregate/WA_LTC_Spending_Biennia.csv</code>).	Both programs show periods of rapid provider or spending growth. Growth alone is not a review-priority indicator; it is the context in which oversight capacity should be assessed.
Network Visibility	Limited , per Minnesota audit findings. Investigators described shared registered agents, P.O. boxes, and family networks obscuring common ownership.	Limited . Washington's public corporate registry (SOS CCFS) does not expose full beneficial ownership, and the public AFH roster does not include related-party data. A bounded proxy (shared phone numbers) identifies 72 multi-home clusters in the three pilot counties; this proxy indicates shared contact information, not verified common ownership.	Both states' public records leave related-party ownership only partially visible. This is a data-availability gap common to public licensing systems, not a finding specific to Washington operators.
Oversight Analytics	Complaint-driven , per the Minnesota Office of the Legislative Auditor, which found DHS relied on complaints rather than systematic claims analytics.	Complaint-driven , per the Washington State Auditor's 2021 performance audit, which found HCA's Division of Program Integrity relied on complaints and lacked systematic risk-based analytics.	Both oversight bodies' own auditors describe a similar reliance on reactive, complaint-driven review rather than routine claims analytics. This is a documented similarity in oversight <i>process</i> , drawn from each state's own audit findings — not a claim that Washington has experienced, or is at risk of, the same outcome as Minnesota.
Billing Controls	Weak , per DOJ and state enforcement filings describing billing for deceased clients, overlapping-hours billing, and out-of-state billing.	Not publicly observable . ProviderOne claims data is not public, so Washington's billing controls cannot be assessed from public records at all.	This is the central data gap this project documents: public records can describe licensing, enforcement, and complaint activity, but cannot show whether Washington's billing controls resemble Minnesota's. Closing this gap requires claims-informed validation with appropriate

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authorization, not public-data inference.

Summary. Washington's AFH licensing model differs structurally from Minnesota's Housing Stabilization Services in ways that plausibly reduce exposure to the specific low-barrier registration pattern documented in Minnesota. Separately, each state's own oversight auditors have described a similar reliance on complaint-driven review rather than systematic analytics — a structural similarity in process, not an empirical finding about outcomes. Public records cannot determine whether Washington's claims-level billing controls resemble Minnesota's, because Washington's claims data is not public. That question can only be answered through claims-informed review conducted by state or federal authorities with appropriate access, and any such review would be subject to ordinary standards of evidence and due process before any conclusion about a specific provider is reached.